Outlook

RE: Policy ownership

From Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Date Thu 29-Aug-24 15:21
To Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>; McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>

Thank you Kevin,

I appreciate you taking my call and listening to my concerns. It means a lot to me. I appreciate you sharing your concerns. I do take your concerns seriously and I am committed to helping this organization improve based on the many concerns that come my way.

I fully agree that "policy should be promptly updated to reflect the new process whenever a process is changed as part of a performance improvement effort."

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ
Chief Quality Officer
Quality Division
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Indian Health Service

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IHS Mission: *To raise the physical, mental, social, and spiritual health of American Indians and Alaska Natives to the highest level*

PIMC Ascend: *Building our future together by listening, empowering, and caring for our Community*

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Thursday, August 29, 2024 2:45 PM
To: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>; McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: Re: Policy ownership

As a follow-up to our conversation and the statement you made regarding policy not being updated when the process changes. I wish to respond in writing with what I feel is Evidence-Based Response :
Processes should not be changed during performance improvement initiatives without concurrently updating the corresponding hospital policies.

Here's why:

1. **Alignment with Best Practices:** Policies and procedures are established to ensure that best practices are consistently applied across the hospital. If processes are changed without updating the policies, there can be a disconnect between what is practiced and what is documented, leading to inconsistency in care and operations.
2. **Legal and Regulatory Compliance:** Hospital policies are often aligned with legal and regulatory requirements. Changing processes without updating policies could result in non-compliance with these requirements, which may expose the hospital to legal liabilities, fines, or accreditation risks.

3. **Clarity and Consistency for Staff:** Policies provide a clear guide for staff on how to perform their duties. If processes are changed but policies remain outdated, staff may be confused about which procedures to follow, potentially leading to errors, inefficiencies, or safety risks.
4. **Accountability and Documentation:** Policies serve as an official record of the hospital's procedures. If a process is changed without updating the policy, there is no documented accountability for the new process, which can be problematic during audits, inspections, or incident investigations.
5. **Effective Communication:** Updating policies to reflect changes in processes ensures that all staff are informed and trained on the new procedures, leading to smoother implementation of improvements and better outcomes.

In summary, the corresponding hospital policy should be promptly updated to reflect the new process whenever a process is changed as part of a performance improvement effort. This ensures that the hospital maintains compliance, consistency, and clarity in its operations.

Supporting Evidence:

The Joint Commission

- **Reference:** The Joint Commission. (2021). *Comprehensive Accreditation Manual for Hospitals (CAMH)*.
- **Relevant Section:** LD.04.01.01, EP 2 – "The hospital complies with law and regulation."
- **Page:** 291.
- **Explanation:** This section emphasizes the importance of hospitals adhering to policies that are in line with regulations, ensuring that practices reflect current laws and standards.

Centers for Medicare & Medicaid Services (CMS)

- **Reference:** Centers for Medicare & Medicaid Services. *State Operations Manual, Appendix A* – Hospital Interpretive Guidelines.
- **Relevant Section:** Tag A-0043 (42 CFR 482.12) – "The hospital must have an effective governing body legally responsible for the conduct of the hospital as an institution."
- **Page:** A-0043.
- **Explanation:** This section discusses the requirement for hospitals to maintain up-to-date policies that align with current practices to ensure compliance with federal regulations.

American Nurses Association (ANA)

- **Reference:** American Nurses Association. (2015). *Nursing: Scope and Standards of Practice* (3rd ed.).
- **Relevant Section:** Standard 5A – "Coordination of Care."
- **Page:** 67.
- **Explanation:** This standard underscore the need for policies to be aligned with practice to ensure safe and effective patient care, highlighting the importance of updating policies when processes change.

Institute for Healthcare Improvement (IHI)

- **Reference:** Institute for Healthcare Improvement. (2017). *Sustaining Improvement*.
- **Relevant Section:** "Embedding Changes in Practice" – "For improvements to be sustained, they must be integrated into the daily work of the organization through updated policies, procedures, and practices."
- **Page:** 12.
- **Explanation:** IHI emphasizes that for performance improvements to be effective long-term, they must be reflected in updated policies and procedures.

V/r

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
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"The single greatest impediment to error prevention in the medical industry is that we punish people for making mistakes." Dr. Lucian Leape

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From: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Date: Thursday, August 29, 2024 at 13:51
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: RE: Policy ownership

Please call me so we can discuss this. Via Teams or 602-581-6563.

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ
Chief Quality Officer
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From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Thursday, August 29, 2024 1:49 PM

To: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Subject: Re: Policy ownership

I must express my deep concern regarding the dismissiveness, disdain, and insulting nature of the insinuation that I am jumping to conclusions. My assessments are far from hasty judgments; they are based on my responsibilities as the Policy and Procedure Officer, where my primary duty is to ensure our organization remains compliant and secure.

Lt. Valdisera's statement clearly indicates that a significant operational change—the transfer of the dosimeter program to Radiology—was executed without the necessary policy update. This omission is not a minor oversight; it directly supports the concern of willful neglect. When such changes are made without corresponding updates to the policy, it exposes our organization to significant compliance risks and potential legal consequences. My role is to ensure that we strictly adhere to these guidelines, and failing to do so represents a critical lapse in our obligation to maintain a safe and compliant environment.

Furthermore, I must address the broader issue of psychological safety within our organization. You have consistently spoken about the importance of psychological safety, yet your actions, such as dismissing concerns with statements like "jumping to conclusions" or devaluing thorough assessments as "too long to give it time," contradict this commitment. These responses do not foster an environment where staff feel safe to raise important issues; instead, they create an atmosphere of fear and hesitation, undermining the culture of openness and safety that we should be striving to cultivate.

I strongly urge that we address these issues head-on. We must update the relevant policy immediately to reflect the operational changes.

Additionally, there needs to be a realignment of our actions with our stated commitment to psychological safety, ensuring that every concern raised is treated with the respect and seriousness it deserves.

V/r

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"The single greatest impediment to error prevention in the medical industry is that we punish people for making mistakes." Dr. Lucian Leape

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From: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Date: Thursday, August 29, 2024 at 13:13
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Subject: RE: Policy ownership

Kevin,

You are jumping to conclusions that data do not support. The policy change is specifically designed to transfer ownership to the radiology department for the exact critical importance you are bringing up. They are the experts. If they lack the training or certification, then they need to acquire that training and certification. They need to own this. EHS officers are not the experts and do not have the expertise for this. The whole purpose of this shift is to address the safety issues and compliance issues that have been identified. This is not willful negligence, it is acknowledgement of safety issues and compliance issues and taking steps to correct them.

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ
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From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Thursday, August 29, 2024 12:19 PM
To: Valdisera, Paula (IHS/PHX/PIMC) <Paula.Valdisera@ihs.gov>; Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>; Numkena, Brian G (IHS/PHX) <Brian.Numkena@ihs.gov>
Cc: Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>; McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: Re: Policy ownership

I acknowledge that we previously depended on the NIH's Radiation Safety Officer (RSO), which may no longer be a viable option following their departure from campus. This development presents a significant challenge to our current RSO designation and our compliance with established policies.

While the Indian Health Manual's definition of the RSO is cited verbatim, this does not negate the critical importance of having a properly trained individual in the role. Adhering to the definition is important; however, it is equally essential that the appointed RSO possesses the requisite training and qualifications to fulfill their duties effectively. Compliance with both the definition and training requirements is crucial to meeting regulatory standards and protecting our organization from potential legal and safety issues.

If we do not have a trained individual to assume the RSO role, this presents an additional legal and regulatory challenge that requires immediate attention. Failure to designate a qualified individual exacerbates our current non-compliance and exposes the organization to increased liability and potential regulatory penalties. It is imperative that we either provide the necessary training to a suitable candidate or seek external expertise to fulfill this role in order to avoid further legal and regulatory complications.

Your statement, "Since early this year, the dosimeter program has been turned over to radiology to facilitate with us assisting in the transition," suggests that the agency may be engaging in willful negligence. This is a serious concern, particularly if this transition was implemented without proper policy updates or ensuring that responsibilities align with regulatory requirements.
It is deeply concerning that we have knowingly remained out of compliance with Policy 9897096 (Radiation Safety and Monitoring). This policy clearly assigns responsibility for radiation safety, including the issuance and tracking of dosimeters, to the Environmental Health and Safety (EHS) department, with RSO duties explicitly designated to the Safety and Occupational Health Specialist at the Phoenix Service Unit. The failure to update or amend the policy following organizational changes earlier this year has exacerbated our non-compliance, effectively placing us in a position of willful negligence.

Willful negligence is a matter of grave concern, as it can have significant and far-reaching consequences, including criminal and civil liabilities for the organization. To mitigate the associated risks, it is imperative that we address and rectify these issues without delay.

Federal, Arizona, and Phoenix regulations regarding the requirement for an RSO are stringent and underscore the importance of having a qualified individual in this critical role to ensure compliance with safety standards and regulatory requirements.

- **Federal Regulations:** Under 29 CFR 1910.1096, OSHA mandates that employers handling ionizing radiation must implement measures to ensure occupational dose limits are not exceeded, supply appropriate personal monitoring equipment, and provide proper training and instruction to personnel. The RSO plays a key role in managing these responsibilities, and their qualifications are typically governed by the NRC under 10 CFR 35.50, which outlines the necessary training and experience for RSOs involved in the medical use of byproduct materials. The RSO must be certified by recognized boards such as the American Board of Radiology or the American Board of Health Physics, and they must have appropriate experience in radiation safety.
- **Arizona State Regulations:** Arizona's requirements are detailed in Ariz. Admin. Code § R9-7-1120, which mandates that every registrant using ionizing radiation must have a designated RSO responsible for implementing and overseeing the radiation safety program. The RSO must meet specific minimum requirements, including formal training, hands-on experience (at least 2,000 hours), and the ability to manage a radiation safety program effectively. If an individual lacks the standard qualifications, the registrant must provide a detailed description of the individual's training and experience to the Arizona Department of Health Services for approval.
- **Phoenix Local Regulations:** Local regulations in Phoenix adhere to broader state and federal guidelines. The Arizona Radiation Regulatory Agency (ARRA) oversees the enforcement of these rules within the state, ensuring that organizations in Phoenix comply with radiation safety standards. The ARRA mandates the qualifications and responsibilities of RSOs, emphasizing the need for continuous oversight and compliance with established procedures to protect public health.
-

In summary, both federal and state regulations require a qualified RSO to ensure compliance with radiation safety standards. It is critical for organizations to designate an RSO who meets the necessary qualifications, as failure to do so can result in significant legal and regulatory consequences.

I also suggest including the Risk Manager to assess and mitigate the potential legal, financial, and operational risks associated with this issue. Additionally, as this appears to be more of a compliance issue, I would also included the Compliance Officer in this conversation.

V/r

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
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From: Valdisera, Paula (IHS/PHX/PIMC) <Paula.Valdisera@ihs.gov>
Date: Thursday, August 29, 2024 at 11:18
To: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>, Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>, Numkena, Brian G (IHS/PHX) <Brian.Numkena@ihs.gov>
Cc: Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>, McCabe, Endreya (IHS/PHX) <Endreya.McCabe@ihs.gov>
Subject: Re: Policy ownership

I will leave Mr. Pope to lead this discussion, but we had discussed this with Phoenix area office earlier also who indicates we follow the Indian health manual which is where I pulled the definitions. No one in EHS meets the criteria for RSO. Since early this year, the dosimeter program has been turned over to radiology to facilitate with us assisting in the transition. I would advise looping them in on this since we follow their guidance.

Paula M. Valdisera, MPS, MPH, CSP, CPH, REHS/RS
LT, U.S. Public Health Service
Safety & Occupational Health Officer
Environmental Health & Safety
Phoenix Indian Medical Center

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From: "Turnage, Kevin D. (IHS/PHX)" <Kevin.Turnage@ihs.gov>
Subject: Policy ownership
Date: 29 August 2024 11:11
To: "Pope, Clinton (IHS/PHX/PIMC)" <Clinton.Pope@ihs.gov>, "Numkena, Brian G (IHS/PHX)" <Brian.Numkena@ihs.gov>
Cc: "Valdisera, Paula (IHS/PHX/PIMC)" <Paula.Valdisera@ihs.gov>, "Holiday, Charisse (IHS/PHX)" <Charisse.Holiday@ihs.gov>, "McCabe, Endreya (IHS/PHX)" <Endreya.McCabe@ihs.gov>

Dear Clint,

Thank you for including me in this critical discussion. I appreciate the opportunity to contribute to ensuring that our radiation safety policies are comprehensive and compliant with all necessary regulations.

After reviewing LT Valdisera's updated draft policy, several key issues and discrepancies have been identified, including conflicts in RSO designation, responsibilities for the Radiation Monitoring Program, and dosimetry badge monitoring. The current and draft policies differ significantly in terms of department roles and responsibilities, which could affect compliance and safety. The RSO's designation must be in writing, meet specific qualifications, and be properly documented to ensure regulatory compliance. A reassessment of these policies is necessary to align with regulatory mandates and maintain safety standards across departments.

I have identified several key issues and discrepancies that I feel need to be addressed to ensure that the policy is comprehensive and compliant with regulatory mandates.

Gaps in Current Policies:

1. **Policy 9897096 (Radiation Safety and Monitoring):**
   - This policy assigns RSO duties to the Safety and Occupational Health Specialist, which conflicts with the draft policy's suggestion that the RSO should be a member of the medical imaging team or a department supervisor. Additionally, this policy places responsibility for the Radiation Monitoring Program under the Environmental Health and Safety (EHS) department, while the draft policy shifts this responsibility to the Radiology Department. These discrepancies indicate a need to reassess the proper delegation of duties and ensure that the RSO is qualified and appropriately positioned within the organization.
2. **Policy 13677097 (Pathology/Laboratory Radiation Safety Policy):**
   - This policy needs comprehensive guidance on radiation safety measures, particularly in defining the qualifications, roles, and responsibilities of the Radiation Safety Officer (RSO), and it needs to adequately address procedures for radiation monitoring and safety training. These gaps could lead to non-compliance and potentially compromise the safety of our personnel.
3. **Policy 15289088 (Dental Radiation Safety Policy):**
   - The policy references the "Phoenix Service Unit Radiation Safety and Monitoring Policy," which currently does not exist or is not accessible. This creates potential compliance issues and may leave staff without guidance to ensure proper radiation safety practices.

Discrepancies and Inconsistencies Identified:

1. **RSO Designation:**
   - The current policy assigns the RSO duties to the Safety and Occupational Health Specialist. In

contrast, the draft policy suggests the RSO should typically be a member of the medical imaging team or a department supervisor when not appointed by the CEO. This discrepancy indicates a potential need to reassess the appropriate individual or department for the RSO role. While the policy is not required to outline who the RSO is, there must be an official delegation, and this information must be easily accessible by staff. It is crucial that the agency delegates the RSO in writing and completes the Arizona Department of Health Services ARRA-DOA Delegation of Authority to ensure compliance.

2. **Administration of Radiation Monitoring Program:**
    - The current policy assigns the administration of the Radiation Monitoring Program to the Environmental Health and Safety (EHS) department, while the draft policy assigns this responsibility to the Radiology Department. This change could impact the program's management and accountability.
3. **Dosimetry Badge Monitoring Responsibilities:**
    - Responsibilities for dosimetry badge monitoring shift between the current and draft policies. The current policy places these responsibilities under the EHS Officer, while the draft policy involves the Radiation Safety Officer and the Radiology Department Manager more directly.
4. **Roles of EHS Officers:**
    - The draft policy alters some responsibilities from EHS Officers to department-specific management, particularly in the Radiology Department, which may affect the chain of command and responsibilities for radiation safety.
5. **Training and Competency Requirements:**
    - The current policy for training and competency relies on departmental managers and EHS, but the draft replaces this with a more collaborative approach involving the Radiology Department and the RSO, potentially affecting how training is monitored.

Required Qualifications for the RSO:

To ensure that the Radiation Safety Officer (RSO) is fully qualified to manage the complexities of radiation safety, the following qualifications should be met, consistent with the guidelines from **10 CFR 34.42**, **10 CFR 35.50**, **NRC**, **VHA Directive 1105**, **AAPM Reports 121 and 160**, and the **Health Physics Society (HPS)**:

- **10 CFR 34.42 (Radiation Safety Officer for Industrial Radiography)**:
    - The RSO must have:
        - **Training and Experience**: Formal training in radiation safety and at least 2 years of practical experience in radiation protection activities related to industrial radiography.
        - **Certification**: Certification as a radiographer by a recognized certifying body is typically required.
        - **Responsibilities**: The RSO is responsible for implementing the radiation protection program, ensuring regulatory compliance, and managing the safety of operations involving the use of radioactive materials and equipment.
- **10 CFR 35.50 (Radiation Safety Officer for Medical Use of Byproduct Material)**:
    - The RSO must have:
        - **Certification**: Certification by a recognized board such as the American Board of

Radiology (ABR), American Board of Nuclear Medicine (ABNM), or American Board of Health Physics (ABHP).
    - **Education and Experience**: A degree in physical science, engineering, or a related field with additional training and experience in radiation safety applicable to the medical use of byproduct material.
    - **Responsibilities**: The RSO must ensure that byproduct material is used safely, manage the radiation safety program, and ensure compliance with all applicable NRC regulations.
- **Nuclear Regulatory Commission (NRC) Qualifications**:
  - The RSO must have sufficient training and experience to effectively oversee the radiation safety program. This typically includes:
    - **Education**: A degree in physical science, engineering, or a related field.
    - **Certification**: Certification from boards like the American Board of Radiology (ABR) or American Board of Health Physics (ABHP) may be required depending on the specific license.
    - **Experience**: The RSO must have practical experience with the types of radioactive materials and equipment used within the facility, including knowledge of NRC regulations and radiation protection principles and the ability to implement and maintain a radiation safety program that complies with NRC requirements.
    - **Training**: The RSO should have completed specialized training programs recognized by the NRC, covering radiation safety, regulatory compliance, and emergency procedures.
- **VHA Directive 1105**:
  - **Educational Background**: The RSO should have a degree in health physics, medical physics, radiologic technology, or a closely related field. Advanced degrees (e.g., MS or PhD) are highly preferred.
  - **Experience**: Significant experience in radiation protection, particularly in healthcare settings, including managing radiation safety programs and conducting radiation surveys.
  - **Authority and Reporting Structure**: The RSO should report directly to the facility's executive management, ensuring appropriate oversight and independence in their role.
- **AAPM Reports 121 and 160**:
  - **Technical Knowledge**: Detailed knowledge of radiation physics, radiobiology, and radiation protection instrumentation.
  - **Certification**: Certification by recognized boards such as ABR or ABHP.
  - **Responsibilities**: The RSO is responsible for overseeing the entire radiation safety program, ensuring compliance with ALARA principles, and maintaining safety protocols to protect both patients and staff.
- **Health Physics Society (HPS) Guidelines**:
  - **Educational Background**: A degree in health physics or a closely related field.
  - **Certification**: Strongly recommended certification by ABHP or ABR.
  - **Experience**: Experience in managing radiation safety programs, especially in healthcare settings, is essential.

Benchmarking Against Other Federal Hospitals:

To ensure we are maintaining the highest standards, I have included **VHA Directive 1105** as a benchmark for what other federal hospitals require. This directive outlines comprehensive standards and responsibilities for radiation safety officers within the Veterans Health Administration, providing a valuable reference as we align our policies with best practices.

Additional documents that can be used for benchmarking include:

- **VHA Directive 1129**: This directive provides guidelines for radiation protection for machine sources of ionizing radiation, which complements the requirements of the RSO role in managing radiation safety programs.
- **NUREG-1556, Volume 9**: This NRC document provides detailed guidance on radiation safety for medical use programs, including RSO responsibilities. It serves as an essential reference for ensuring compliance with NRC regulations.
- **Health Physics Society Guidelines**: These guidelines offer a comprehensive overview of RSO qualifications and responsibilities, mainly focusing on healthcare settings.

Required Actions:

- The policy must clearly outline the qualifications required for the RSO, ensuring that the individual meets the educational, certification, experience, and authority criteria outlined above.
- Regulatory compliance is crucial. The **Nuclear Regulatory Commission (NRC)** and **OSHA** standards, as well as the **Arizona Radiation Regulatory Agency (ARRA)**, mandate that facilities designate a qualified RSO who meets the state's standards for radiation safety management. These regulations are complemented by VHA guidelines for managing radioactive materials and ensuring safety.
- Any policy ownership updates or transfers must ensure that these regulatory standards are met and that the RSO is delegated in writing with the **ARRA-DOA Delegation of Authority** completed.

A formal written designation is crucial because it provides clear documentation of who is responsible for overseeing radiation safety within the facility. The written delegation typically includes:

1. **Official Appointment**: A document that explicitly names the individual designated as the RSO.
2. **Responsibilities**: A clear outline of the RSO's duties and responsibilities related to radiation safety.
3. **Authority**: A statement of the authority granted to the RSO to implement and enforce radiation safety policies and procedures.
4. **Signature**: The document is usually signed by a senior management official or the highest level of authority within the organization, indicating that the designation is officially recognized and supported.

Just as Phoenix Indian Medical Center is required to have a designated Hazardous Material Coordinator to ensure the safe management and compliance of hazardous materials across the facility, it is equally critical that the hospital has a formally delegated Radiation Safety Officer (RSO). The RSO plays a vital role in overseeing the radiation safety program, ensuring compliance with federal, state, and tribal regulations, and safeguarding the health and safety of our patients, staff, and visitors.

The RSO must be officially appointed and possess the necessary qualifications and authority to manage all aspects of radiation safety within the hospital. This includes implementing safety protocols, monitoring radiation exposure, and responding to any radiological incidents. The delegation of an RSO is not only a regulatory requirement but also a fundamental component of our commitment to maintaining a safe and compliant healthcare environment.

Without an officially delegated RSO, PIMC would be at risk of non-compliance with radiation safety regulations, similar to the risks posed by the absence of a Hazardous Material Coordinator. Therefore, it is imperative that this role is clearly defined, formally delegated, and supported by hospital leadership to ensure the highest standards of safety and regulatory compliance are met.

This written delegation is important for compliance with regulatory requirements and must be easily accessible to staff. It serves as an official record that can be presented to regulatory agencies, such as the Nuclear Regulatory Commission (NRC) or state health departments, during inspections or audits.

In summary, having a designated Radiation Safety Officer is a standard requirement for hospitals with Radiology departments to ensure the safe use of radiation and compliance with regulatory standards.

Since these updates involve policies that span across various departments within our organization, these departments must be included in the conversation for awareness, if nothing else. This will ensure that everyone is informed and aligned with the necessary changes.

I see no issue with the transfer of policy ownership to Radiology, provided these critical points are addressed.

Thank you for your attention to this critical matter.

Best regards,

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™
Nurse Consultant, Policy and Procedure Officer, COR II,
Phoenix Indian Medical Center
Cell: 602-647-9126

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Legal Disclaimer

The analysis, review, and recommendations provided by the Policy and Procedure Nurse Officer at Phoenix Indian Medical Center (PIMC) are conducted as part of the officer's official responsibilities. These actions are intended to ensure that all policies and procedures within PIMC are thorough, consistent, and compliant with applicable federal, state, and tribal regulations, as well as Indian Health Service (IHS) standards and industry best practices.

The information and recommendations provided by the Policy and Procedure Nurse Officer are intended to support PIMC's commitment to maintaining a safe, compliant, and high-quality healthcare environment. However, this content should not be interpreted as legal advice. PIMC is encouraged to seek guidance from legal counsel or appropriate regulatory authorities to confirm that all policies and procedures fully comply with legal and regulatory requirements.

The Policy and Procedure Nurse Officer does not assume responsibility for the legal adequacy of the policies and procedures reviewed or developed. Phoenix Indian Medical Center retains ultimate responsibility for ensuring that all policies and procedures meet the necessary legal, regulatory, and industry standards.

From: Pope, Clinton (IHS/PHX/PIMC) <Clinton.Pope@ihs.gov>
Date: Thursday, August 29, 2024 at 09:18
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Cc: Valdisera, Paula (IHS/PHX/PIMC) <Paula.Valdisera@ihs.gov>, Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>, Holiday, Charisse (IHS/PHX) <Charisse.Holiday@ihs.gov>
Subject: Policy ownership

Hi Brian,

We need to transfer ownership of the "Radiation Safety and Monitoring" policy to you or someone in Radiology. LT Valdisera made updates to a draft that is currently in PolicyStat (also attached). As the policy owner, please edit it as needed to manage radiation safety and monitoring to keep everyone safe.

What questions do you have for me? What concerns do you have?

Thanks,

Clint

Very Respectfully,

Clinton D. Pope, MSIE, CPHQ
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IHS Mission: *To raise the physical, mental, social, and spiritual health of American Indians and Alaska Natives to the highest level*

PIMC Ascend: *Building our future together by listening, empowering, and caring for our Community*